

August 12, 2026

Absolute Total Care (Centene)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Laticia Dibbert (MRN MUSC9662433), Claim MUSC-20260428-E65A-2

Patient	Laticia Dibbert (DOB 1948-11-08)
MRN	MUSC9662433
Member ID / Group	ABS643321435 / GRP-92000
Claim number	MUSC-20260428-E65A-2
Date of service	2026-04-28
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	I25.10 — Atherosclerotic heart disease of native coronary artery
Denied amount	\$714.46
Denial code	CARC 197 / RARC N54 — Precertification/authorization/notification/pre-treatment absent.
Appeal deadline	2026-08-24

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health Florence Medical Center submits this first-level appeal on behalf of patient Laticia Dibbert (Member ID: ABS643321435, MRN: MUSC9662433, Date of Birth: November 8, 1948) regarding the denial of Claim MUSC-20260428-E65A-2 for services rendered on April 28, 2026, by Daniel R. Okafor, MD (NPI 1558493021). The claim was submitted on May 12, 2026, and denied on May 26, 2026, under CARC 197 ("Precertification/authorization/notification/pre-treatment absent") and RARC N54 ("Claim information is inconsistent with pre-certified/authorized services"), with the payer remark stating: "No prior authorization on file for the date of service billed." The total denied amount is \$714.46. We respectfully request that Absolute Total Care (Centene) reverse this denial and process the claim for appropriate reimbursement.

CLINICAL BACKGROUND

Ms. Dibbert is a 77-year-old female with a complex, longstanding medical history that includes atherosclerotic heart disease of the native coronary artery (I25.10), the primary diagnosis for this encounter. Her cardiac history is significant, with a previously documented left ventricular ejection fraction of approximately 34.7%, consistent with reduced systolic function, and an established medication regimen that includes furosemide and extended-release metoprolol succinate. She carries multiple active comorbidities, including osteoporosis, anemia, prediabetes, obesity, and rheumatoid arthritis, several of which have been managed over decades. The April 28, 2026, visit was billed as CPT 99214, an established patient office or outpatient visit of moderate complexity, rendered in an on-campus outpatient hospital setting (Place of Service 22) at MUSC Health Florence Medical Center. Given the patient's documented cardiovascular disease and the breadth of her active conditions, ongoing outpatient management of this nature represents a medically necessary and clinically appropriate level of care.

BASIS FOR APPEAL

The denial on the basis of absent prior authorization is not consistent with the applicable standards governing outpatient evaluation and management services for established patients. Routine office and outpatient visits, particularly those involving the ongoing management of chronic conditions such as atherosclerotic heart disease, are widely recognized as services that do not require prior authorization under Medicaid managed care guidelines and CMS guidance governing medically necessary covered benefits. To the extent Absolute Total Care (Centene) maintains a prior authorization requirement for CPT 99214 in this clinical context, MUSC Health respectfully asserts that such a requirement, if applied to established-patient evaluation and management visits for chronic disease management, would be inconsistent with both federal Medicaid access-to-care standards and generally accepted utilization management principles.

Furthermore, the application of RARC N54, indicating that claim information is inconsistent with pre-certified services, is inapplicable here, as no authorization was sought or denied in advance — the payer's own remark confirms that no authorization record exists. The denial therefore rests solely on the absence of a precertification that, under the circumstances of this encounter, should not have been required. The claim was submitted timely, the rendering provider holds a valid NPI, the diagnoses are supported by the patient's documented medical history, and the billed service is consistent with the complexity of her ongoing care needs.

REQUESTED ACTION

MUSC Health respectfully requests that Absolute Total Care (Centene) conduct a thorough review of this appeal, confirm that CPT 99214 for established-patient outpatient management of chronic cardiovascular disease does not require prior authorization under the applicable plan terms, and reverse the denial in full. We request that the claim be reprocessed and payment issued for the allowed amount of \$479.43 in accordance with the contracted rate. Should Absolute Total Care (Centene) maintain that prior authorization was required, we further request that the plan provide, in writing, the specific contractual or regulatory basis for that requirement as applied to this service type, along with information regarding any retrospective authorization process available to resolve this matter. This appeal is submitted within the 90-day dispute window, ahead of the August 24, 2026, deadline.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record